

Interview Transcript with Dr. Jonas Fischer (Hamburg, Germany) Emergency Medicine Specialist, UKE Hospital

Interviewer: Dr. Fischer, thank you for joining us. Could you start by describing your background and role during the pandemic? Dr. Fischer: Of course. I'm Dr. Jonas Fischer, an emergency medicine specialist at University Medical Center Hamburg-Eppendorf, one of northern Germany's largest hospitals. Before 2020, I primarily worked in trauma and cardiac emergency care—accidents, heart attacks, strokes.

When COVID-19 struck, the emergency department became the gateway to the pandemic. We had to reinvent how we triaged, stabilized, and even thought about patients. For the first time in my career, the emergency room wasn't just about seconds and minutes—it was about containment, coordination, and compassion at scale.

Interviewer: How did the emergency department adapt when the first wave began? Dr. Fischer: The transformation was immediate and drastic. By late February 2020, we had our first suspected cases arriving from northern Italy. Within 72 hours, our ER was divided into two parallel systems—a COVID zone and a clean zone.

We built physical barriers, re-routed patient flow, and even set up tent triage stations outside the main hospital. Ambulances had to radio ahead with symptom checklists—fever, cough, oxygen saturation—so we could direct them safely.

The hardest part was psychological. We were used to touching patients freely—now everything had to go through gloves, masks, and layers of fear. I remember telling my team, "We'll still save lives—just from behind glass."

Interviewer: What were the biggest operational challenges you faced? Dr. Fischer: The first was triage uncertainty. Early symptoms of COVID mimic everything—flu, asthma, anxiety. Many patients came in terrified but healthy; others walked in talking and collapsed minutes later.

We introduced a dynamic triage protocol using oxygen saturation and breathing patterns rather than just fever. It wasn't perfect, but it helped. The second challenge was overcrowding—not just in beds, but in information. We had to process constant updates from the Robert Koch Institute, city health boards, and global studies—often contradicting one another.

And third, the emotional toll. We're trained for trauma and adrenaline—not for a crisis that lasts two years. The ER became a pressure cooker of exhaustion and empathy.

Interviewer: How did Hamburg's healthcare network handle patient load during the peaks? Dr. Fischer: Hamburg has a relatively integrated hospital system, which helped immensely. We created a citywide emergency coordination cell—linking ambulance dispatch, hospitals, and public health authorities. It was like air traffic control for patients.

If one hospital's ICU filled up, ambulances were rerouted within minutes to another. We also set up mobile ICU teams that could transfer critical patients safely between hospitals. It was logistics as much as medicine.

During the Delta wave, when cases surged beyond capacity, we even received patients from other German states. The spirit of cooperation was remarkable—doctors from Lübeck, Bremen, and Kiel worked alongside us as one team.

Interviewer: Did you experience shortages of equipment or personnel? Dr. Fischer: Yes, but not always in the ways people imagine. We rarely ran out of ventilators—Germany had many. What we lacked were trained hands to operate them. Nurses and paramedics worked double shifts for months. Many got sick themselves.

In early 2021, when cases spiked, we had to call in retired clinicians and even military medics to support triage. The Bundeswehr helped set up field units near Hamburg Airport. That collaboration was lifesaving.

Emotionally, though, the shortage was of rest and reassurance. You can restock PPE; you can't restock morale.

Interviewer: What was the most difficult ethical situation you encountered? Dr. Fischer: Triage under pressure. There's one night I'll never forget—April 2021. We had three ambulances arriving simultaneously, all with critical COVID patients. Two ventilators left.

We had to make immediate decisions based on likelihood of survival. We called in a second consultant to cross-check decisions. In the end, one patient—a 73-year-old with severe comorbidities—was managed palliatively because the ventilator went to a younger man in cardiac distress.

That choice still sits heavy with me. As emergency doctors, we're trained to fight for every heartbeat. COVID forced us to measure mercy against mathematics.

Interviewer: How did you and your team cope emotionally with such moments? Dr. Fischer: We tried to build small rituals of resilience. After every code blue—every resuscitation—we'd pause for 30 seconds of silence. No words, just breathing together.

We also started peer support circles every Friday—informal gatherings to vent, cry, or share dark humor. Sometimes we invited a psychologist, but mostly we helped each other. One night, after losing two patients in a row, our nurses spontaneously sang "You'll Never Walk Alone." In that moment, we weren't staff—we were a family.

Interviewer: How did vaccination change your work in the ER? Dr. Fischer: It changed the case mix overnight. Before vaccines, nearly every severe patient was unvaccinated. By mid-2021, vaccinated cases became rarer and less critical. That gave us breathing room.

We also turned part of the ER into a vaccination station for walk-ins. Seeing hope literally

walk through the same doors that had seen so much fear was healing.

However, misinformation was rampant. I remember a man in his 40s refusing vaccination, later arriving cyanotic and terrified. He survived, barely. When he recovered, his first words were, I wish I had listened. I think of him often not with anger, but sadness.

Interviewer: What role did technology play in managing emergency care? Dr. Fischer: A huge one. Hamburg developed a digital dashboard tracking every hospital's real-time bed availability, oxygen status, and staffing. We could see, at a glance, where to send patients. That system now serves as a model for other emergencies.

We also used telemedicine to consult rural hospitals on ventilation protocols. COVID pushed emergency medicine into the digital era. I believe these systems should become permanent pandemics aren't the only crises we'll face.

Interviewer: How did public cooperation evolve over time in your view? Dr. Fischer: The first wave brought unity—applause from balconies, donations of food. But by the second year, fatigue and polarization took over. We had protests outside hospitals, even harassment of staff over vaccine mandates. That was painful.

Still, for every critic, there were ten silent supporters. Ordinary people sent thank-you notes, hand-knitted masks, or flowers. Those small gestures mattered more than they'll ever know.

Interviewer: Looking back, what did Germany's emergency response do right—and what could improve? Dr. Fischer: We did three things right: 1. Rapid reorganization—creating separate COVID and non-COVID pathways saved countless lives. 2. Inter-hospital coordination—the digital dashboards and patient routing worked brilliantly. 3. Evidence-based adaptability—our protocols evolved with science, not politics.

What we could improve: long-term workforce sustainability. Emergency medicine runs on adrenaline, but pandemics require endurance. We need better rotation systems, psychological support, and incentives to retain staff post-crisis.

Interviewer: Finally, what has this pandemic taught you personally? Dr. Fischer: It taught me that heroism is quiet. It's not dramatic rescues—it's showing up every day, exhausted, and doing your duty anyway.

I learned that medicine is both a science of precision and a practice of empathy. Numbers save systems, but kindness saves souls.

And perhaps most importantly, I learned humility. In the ER, we like control—clear diagnoses, clear outcomes. COVID shattered that illusion. But it also reminded us why we chose this path to stand in chaos and still offer calm.

Interviewer: That's profoundly said, Dr. Fischer. Thank you for your service and your insight.

Dr. Fischer: Thank you. I only hope we carry the lessons forward because the next emergency is never far away.

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